OSCE Clinical Reasoning & Bias Evaluation Rubric

Standardized Patient Simulation Framework | Assessing Cognitive, Diagnostic, and Interpersonal Biases

> **Instructions for Evaluators & Virtua SP Platform:** This rubric is designed to assess medical students during complex standardized patient encounters (e.g., Denise Vance case: acute chest pain with comorbid GAD and triage framing risks). Score students across the five core cognitive error categories using the 1–4 scale below.

1. Structured Competency Matrix

Bias Category	Target Behaviors & Failure Modes	Unsatisfactory (1)	Developing (2)	Proficient (3)	Exemplary (4)
Premature Closure & Anchoring	Locks onto initial diagnosis early; ignores evolving clinical data or physical exam findings.	Fixates on initial impression; fails to adjust reasoning.	Considers alternates only when explicitly prompted.	Actively updates differential as new data emerge.	Performs diagnostic time-outs; systematically revises differential.
Confirmation & Search Bias	Asks leading questions supporting the leading hypothesis; skips disconfirming evidence.	Asks leading questions; ignores contradictory symptoms.	Asks mostly confirming questions passively.	Asks balanced positive and negative predictive questions.	Probes for key negatives and red flags ruling out alternatives.
Implicit Demographic Bias	Assumptions based on race, gender, age, socioeconomic status, or mental health history.	Uses dismissive language; attributes symptoms solely to lifestyle.	Acknowledges social determinants superficially.	Maintains objective clinical standards; non-judgmental.	Actively counteracts bias; addresses social determinants equitably.
Framing & Availability	Influenced by sign-out framing, recent patient encounters, or emotional recall.	Accepts handoff label uncritically without baseline inquiry.	Mentions framing but struggles to decouple from case.	Distinguishes current presentation from previous cases.	Identifies framing risks; evaluates from first principles.
Diagnostic Attribution	Attributes complex somatic complaints entirely to psychiatric history or chronic illness.	Dismisses new symptoms as psychological without workup.	Investigates minimally before attributing to baseline.	Thoroughly evaluates acute complaints independently.	Rigorously separates acute pathology from baseline history.